

Meeting Transcript — September 4, 2026

Participants: Isak Gutierrez, Thomas Abshier, Joelle Ater, Leonard (CAD/manufacturing, joined ~9:22 AM) **Duration:** ~4 hours (7:03 AM – 10:57 AM)

Part 1 — Classification & Medicare Discussion (~7:03–7:30)

Joelle: Was this question already answered — because it has the lift capability?

Isak: Yeah. Let me see where we landed on that.

Thomas: I think the reason for making it a medical device is that we want to leave open the path for Medicare and Medicaid.

Joelle: I thought that was also eliminated, though.

Isak: Medicare is typically for 65 and up...

Thomas: That was a conversation your dad and I had — about whether you can get somebody actually out of bed. And if that's the only reason you can make this a Medicare thing, you actually get them out of bed. I asked your dad, and he wrote back about Jeanette. Her father had a stroke and was in bed, and they were able to set him up — just got him on the edge of the bed, and after that they were able to use the LiftWalker to get him completely around. So this did everything that a Hoyer lift did. The only requirement was they were able to get somebody to have their legs dangled off the edge of the bed.

Joelle: So that's a bit of an outlier case. Not that I want to exclude them, but I'm wondering if we need to... Okay, not advertising to them doesn't mean they couldn't use it. It would just mean that they couldn't get it covered by Medicare or Medicaid if it wasn't technically a medical device, correct?

Thomas: Yes. We're not going to be... if we don't try and do something like this, we will never have Medicare. I don't want to just eliminate that one. That one's the gravy train. That's the 200-car freight train of sales.

Joelle: Was it eliminated, though, as a possibility? Medicare?

Thomas: I don't think so. That's the question. Claude said yes, it's eliminated — don't even think about it. And I'm not... after your dad's story about being able to actually get people up. All you have to do is get their legs off the edge — here's a floppy guy, couldn't walk, he didn't even have a back strap, and we were able to make it work. Now we have something better than then.

The real question is: can pretty much every bed-bound patient get their legs off the edge of the bed? And the answer is probably. Once you do that, you've removed the indignity of the bedpan. You've restored them to be able to do normal toileting. That is a huge win for somebody's life.

Joelle: Was there another reason Medicare was eliminated from the possibility, or was that it?

Isak: So Medicare and Medicaid — Medicaid is going to be a little bit easier to get insurance to cover. I was concerned that we won't get Medicare or Medicaid for whichever classification we pick. We'll have to see. That's an FDA call.

We could have it reimbursed for a couple different things: either this patient needs it because they are unable to move without it and they need something to lift them out of bed and transfer them to the toilet — or this is just a walker, and anybody who wants insurance reimbursement, it has to be used specifically for walking versus “this patient just needs to be transferred and moved around.” That’s a different product.

My concern was that Medicare/Medicaid would only cover one of these or none of them, depending on what we classified it as and what we call it in the copy.

Joelle: From what you sent over, it sounded like if you have two codes, one would supersede the other — the lift would supersede it — but it didn’t seem to negate the other option.

Thomas: That was the question. That’s it right there.

Part 2 — California Manufacturing Problem (~7:10–7:28)

Joelle: Here’s another complication from being classified as a medical device. We technically wouldn’t be able to sell the existing manufactured ones. There wasn’t any regulation — the California regulation needs to happen first, the licensure for manufacturing a medical device.

If we pursue that route later, or start manufacturing a similar device in another state without all the regulations — it just adds a huge layer of complication.

Isak: What does that look like? Are there specific states that would be better?

Joelle: Other states recommended — not just for ease of regulation but for manufacturing availability and cost of labor — Michigan, Minnesota came up, and then Utah, which could be very feasible since it’s easy to get to from California and Idaho.

Thomas: Leonard — we’ve been working with Leonard and he actually already has several contract manufacturers lined up. But as I remember, they were more expensive than Victory Circle, so we went with Victory Circle. But given that if we could unlock the Medicare box, whatever we have to pay is worth it.

Joelle: The cost of time as well. Is there already a manufacturer that is a possibility in Utah?

Thomas: We’ve already sent some of the drawings over to them. The question is whether they are GMP medical qualified or not. That one we did not look at because that wasn’t on the radar.

Thomas: Getting a hold of Leonard would be the fast track. He may already have the quotes.

Part 3 — Selling Existing Units (~7:13–7:18)

Joelle: What do we do with the existing ones? Does dad just sell them as he always has while we pursue the legal route?

Thomas: The idea we talked about yesterday was the adult care foster home thing. It’s possible we could do phone calls into another state and sell to adult care foster home people. We’re not selling it as a medical device — we’re selling it as something to protect them as a worker. That

one doesn't even require a classification at all because we're selling it as an employee protection device.

Joelle: Having two similar devices, one marketed as a walker and one as a lift — is it legal?

Isak: If their bill of goods is different — if they're actually unique products, then yes. But if they're essentially exactly the same except for portability, we're going to have a hard time defending that position.

Joelle: I'm just worried about the legal ramifications. If we're pursuing the medical device route and getting certification in another state, and meanwhile we're trying to sell the same device branded differently — would we need a separate business for that?

Thomas: Your question about whether what we do now sets a precedent for what we have to do later is a really important one. That's probably actually a legal question.

Joelle: What type of lawyer do we need for this?

Part 4 — Non-Device Path Analysis (~7:19–7:21)

Isak: (*reading Claude analysis*) The test isn't the product — it's the claims. Registration is required of anyone who manufactures a "device," and "device" is defined by intended use. Something intended for diagnosis, cure, mitigation, treatment, or prevention of disease, or intended to affect the structure or function of the body.

A treadmill sold for fitness isn't a device. The same treadmill sold for gait rehabilitation is.

To be a non-device, everything below has to go: any mention of disability, mobility impairment, spinal cord injury, stroke, any condition, prescription, clinics, therapists, rehab, hospitals...

It's basically cutting our legs off and then putting us in a walker.

Dad's pitch — "if you've been in a wheelchair, let me show you how to walk" — that's the mitigation claim in plain language.

We'd also give up HSA and FSA eligibility, which requires the item to be medical. Any insurance path, now or later, gets given up.

Part 5 — GMP Requirements (~7:39–7:40)

Isak: (*reading Claude analysis*) What does GMP actually require of a shop building walkers? Mostly organization, not construction: - Calibration of measuring and torque equipment - Segregated areas for incoming, in-process, finished, and non-conforming product - Status tags, controlled label storage - Current work instructions at point of use - Equipment maintenance logs, training records - Component traceability and build record per unit - No clean room, no HVAC controls

Joelle: Sounds a little bit easier than becoming a medical device manufacturer in California.

Part 6 — Leonard Call (~9:22–9:42)

Isak: Hey Leonard, thanks for meeting up. Thomas had said you guys have talked in the past to people, maybe in Utah, about manufacturing, and we ended up going with Victory Circle. We're looking at our options because California is a pretty crazy place to start a medical manufacturing business.

Leonard: Yeah. With these LiftWalkers and wheelchair things, Charlie has designed features that require a shop with a lot of different capabilities — bending those arcs, full weldments, cutting and bending metal.

There's a group here in Spanish Fork called **Oshkut** that can make parts. They have capability to laser cut tubing. They're building up a bender for tubing but it's not online yet. They also do sheet metal — about 8 lasers, all computerized. You can get online quotes really quickly by submitting your parts.

There's another place — **Suncut Sand** — I've had them laser cut some things for me. Pretty good.

There's a wheelchair manufacturer right next to Oshkut. They manufacture a line of lightweight wheelchairs — the mono-frame ones. They also just came out with a motorized off-road wheelchair. Pretty cool. But they don't do contract manufacturing — they just manufacture their own stuff. They purchase parts and do assembly, fulfill orders.

There's a lot of opportunity up here. Manufacturing has really become quite an emphasis for state and city governments — they assist, help, and get out of the way. Huge entrepreneurial movement.

It would be wise to probably not set it up in California. Even though Victory Circle's numbers came in so much lower than any other place. It was amazing how cheap they could make parts.

Joelle: What if Victory Circle does parts and then someone else is the official manufacturer elsewhere, like in Utah?

Leonard: Yeah, or some other place. It doesn't have to be Utah.

Leonard: Another possibility — we could get our parts made overseas. Brought over, do the final assembly, packaging, boxing, shipping here. Still assembled in the US but parts manufacturing... you can't beat the price. Not just China — Malaysia, Thailand, Vietnam, India. All these countries are begging for business.

I have a guy who can source all of these parts for us overseas. He does similar type of products. He's from Nigeria, really nice guy, knows his stuff. His partner lives in Hong Kong but sources everywhere. He would manufacture under his name and sell to us — so we don't have to deal with separate little companies. Very reasonable.

I could send him the Traveler tech package — that's the most developed in terms of documentation.

I'm also working on the Flyer (LiftWalker) right now, trying to get that package together. Hopefully done next week. And I'm building parts for the first brainwave headgear — they're coming together really nice.

Leonard: So I'll do both — search local and talk to some people that have capabilities we need, and have my China guy look at it too.

Part 7 — DME Supplier Deep Dive (~9:42–10:57)

Thomas: After Joelle left — the issue of whether there's anything that's a bridge too far on the medical approach, either Medicare or doing the DME. Both would be ideal. But there might be something that's just too much.

DME Facility Requirements (from Claude analysis)

Isak: *(reading)* No retail showroom required. The regulation explicitly permits a closed-door business. Online-only doesn't work — there's no version that runs from a laptop. But the useful carve-out: locations used solely as warehouses need no separate enrollment, bond, or accreditation. That's a meaningful simplification for a manufacturer.

Full checklist: 200 square feet minimum, public access, staffed during posted hours, signage, record space, minimum 30 hours/week open to public, cannot share location with another Medicare supplier, landline telephone (no cell as primary), must permit unannounced CMS inspections.

Cost Analysis

Isak: One-time roughly \$9,500 to \$25,000. Annual \$50,000 to \$78,000 — staff, rent, compliance, accreditation. Break-even is roughly 30 units a year, about 2.5 a month. Below that, the dealer is cheaper. Above it, self-enrollment starts to pay. Current volume is near zero.

Moratorium

Isak: There was a nationwide DME POS enrollment moratorium February 27 through August 27, 2026. It expired — CMS decided not to extend it. The door is open. But it can come back — the authority runs in 6-month increments.

DME Acquisition

Isak: There's an active market on dmelisting.com. Two types: - Brand new shell companies at \$175K — clean PTAN, zero billing history. But under 36 months, buying majority control triggers fresh enrollment. Defeats the purpose. - Operating businesses at \$300K — e.g., Van Nuys, LA. 15 years operating, licensed, accredited, Medicare + Medi-Cal, 40+ HMO/PPO contracts. Some of those HMOs no longer award contracts to new suppliers.

The acquisition target you want is an old, boring, long-enrolled operator — not a new clean one. Asset purchases don't transfer enrollment at all — must be equity deal.

Thomas: So really, if you're going to buy a business, you have to do all the due diligence on whether this is a real business. Why are they selling it?

DME as Platform

Isak: What this opens up for us is that if there's other products, other lines, other people's stuff — potentially we end up selling other durable medical equipment and becoming a real force.

Thomas: Interesting. I had not thought of that one. That actually does open up an entirely new vista.

Recommendation

Isak: *(reading)* Do not build or buy a DME operation now. Not on cost grounds — on timing grounds. 12 to 18 months before a single billable claim, during which 4 other channels are available and need no code at all. The best way to have a DME is to use someone else's. An enrolled supplier can carry the product next month. That also tests whether the channel produces anything real before \$200K or 18 months goes into owning one.

Thomas: That's a great approach. It comes down to — if we have flow, we can afford the employees. We're in a position where we don't have to make money right now. If we can break even doing what we're doing on this startup, we're fine. But if we can get the infrastructure settled — Medicare, DME, manufacturing — and get a little bit of flow so we're paying for payroll... we're taxiing. We're not lifting off. But we're picking up airspeed. We'll eventually be able to pull back the stick.

Isak: I think we're thinking bigger now, and this is good. It's not outlandish. We're being conscious. I appreciate you giving us this time for research. It feels like we're giving birth to a lot more ideas than "we just need to sell a Walker."

Thomas: Have Claude run a cost for doing Medicare and a cost for establishing a DME.

Isak: Minimum wage in Utah is federal minimum, \$7.25. Just a thought.

Medi-Cal Pricing Analysis

Isak: *(reading)* Medicare publishes fee schedule rates — take it or leave it. For listed codes like E0135, E0149, there are set allowables. Trap is the "least costly alternative" — unjustified features on a walker get paid at E0143 allowance regardless of what the device costs.

For miscellaneous codes, no published rate. It's individual consideration. For codes with no maximum allowable rate, reimbursement is the lowest of several ceilings: usual charges to general public, net purchase price plus 100% markup, acquisition cost plus state markup, a percentage of MSRP documented by catalog, or 80% of Medicare rural fee schedule for same or similar item.

If DHCS deems a lifting walker "same or similar" to a walker code, the ceiling is a few hundred dollars, and no amount of MSRP documentation helps.

Thomas: We're not doing walkers — we're doing lifts. We're totally medical. I think we've gotten around that issue.

Wrap-Up

Thomas: I think the gate is really the DME/Medicare thing. Until we've said yes, we're going to do it, it really does change the course. I think there's a huge payoff. By being a DME, we make it profitable. The place where Medicaid died was because of not being a DME. I have a feeling it's going to be a big deal to be a DME. But I think it might be worth it.

Isak: I think it's the best move. I don't want to do anything right away that prevents us from going that way later. I think we pay the fees and do this process — maybe in California, maybe not. Utah might be a great idea.

Thomas: Make sure there aren't any deal killers. Is there anything that's going to be too much?

Isak: Sometimes Claude makes a decision and takes it as law. Our plans might be changing. I'll find out those questions, organize it all, give Medi-Cal a call, and feel out what they're willing to pay. If I can figure out what they paid for Biodex or something — can't hurt to ask.

Thomas: That's a great idea.

Isak: Reminder — Monday no trading, closed for Labor Day. I'll be out of town for the wedding but I'll be there Tuesday morning on Wi-Fi.